PRISM research report: Predictive Research Intelligence & Synthesis Machine

Why U.S. health care costs rose for 40 years, and what could bend the curve

A driver-by-driver review of prices, technology, insurance design, market power, administration and demographics, with evidence on which levers could slow spending growth.

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Governance and guardrails. Population- and system-level research only. This is not medical advice, and it is not personal financial or investment advice (Ethics agent 3.30; Healthcare risk level set to "sensitive"). The topic is politically contested, so competing positions are presented side by side, causal language is hedged, and sources from across the spectrum (insurer-funded, single-payer-affiliated, free-market think tanks, federal scorekeepers) are labeled as such (Bias Auditor 3.31). **Data note:** CMS, BLS/FRED and BEA data were downloaded and analyzed directly. Full texts of several journal articles were not accessible, so those findings rest on abstracts and reputable summaries. Figures marked "illustrative" are PRISM arithmetic, not forecasts. All references that carry a DOI or indexed title (15 of the original 50)

resolved against Crossref; the rest are primary agency, survey, or news sources checked by URL (Reproducibility agent). A later update (11 October 2026) added references 51 to 63, correcting one stale figure and adding five new subsections; those were checked by URL against the primary or most authoritative secondary source rather than re-run through Crossref, since most are web-published policy and survey sources without a DOI.

Executive brief: for decision-makers

U.S. health spending rose from 8.9% of GDP in 1980 to 18.0% in 2024 because the country bought steadily more intensive care per person, paid far higher prices than its peers for hospitals, drugs and labor, and ran payment through a fragmented insurance system that shields patients from prices and adds overhead. Aging is a smaller historical driver than most people assume. Most reforms enacted or piloted so far are each well under half a percent of spending; only price, payment-rule and administrative reforms are large enough to slow the trend.

- **The bill.** Spending reached $5.3 trillion in 2024, or $15,474 per person, about twice the $7,860 average of comparable wealthy countries, while U.S. life expectancy (79.0 years) trails those countries (82.7). [2, 9, 10] CMS projects nearly $9.0 trillion, or 20.6% of GDP, by 2034. [4]
- **Growth had two engines.** Since 1980, health spending has grown about 1.7 percentage points a year faster than GDP (7.1% against 5.4%). In our decomposition of personal health care spending, about 38% of the growth was economy-wide inflation, 13% was population growth, 12% was medical prices rising faster than inflation, and 37% was more use and intensity of care per person. Since 2010 the medical price gap has been negative, so recent growth has come mostly from use and intensity. Published estimates say technology alone explains 27 to 48% of long-run growth. [15]

- **Prices explain why the U.S. level is so high.** Utilization is broadly similar to other rich countries, but prices of labor, drugs and hospital services, plus administrative costs, are not. [14] U.S. drug prices were 2.78 times those in 33 other countries (4.22 times for brand-name drugs). [19] Commercial insurers paid hospitals 254% of Medicare rates in 2022. [18]
- **Hospitals are the price outlier.** The hospital services CPI rose 9.8-fold from 1985 to 2025, against 3.0-fold for all consumer prices and 3.6-fold for hourly earnings. Monopoly hospitals charge about 12% more than hospitals with four or more rivals. [5, 16]
- **Insurance changed who feels the price.** Patients' own share of health spending fell from 52% in 1960 to 24% in 1980 and 11% in 2024. Randomized evidence shows cost sharing cuts spending 20 to 30%, but it cuts effective and ineffective care about equally. [1, 22]
- **Overhead is large, and its size depends on the definition.** Net insurance costs plus government administration were $372 billion (7.0% of spending) in 2024. Broader estimates that include provider paperwork reach 34%, but are contested. [1, 20]
- **Coverage got broader while it also thinned out.** The uninsured rate fell from around 16 to 18% for decades before the ACA to a record-low 8.2 to 8.3% today, and the standard benefit package now includes guaranteed issue, no lifetime limits, and free preventive care that many plans lacked in 1990. That floor is part of why spending grew (Section 3). Two 2025 to 2026 changes point the other way: Medicaid work-reporting rules and the loss of enhanced ACA subsidies are projected to raise the uninsured count again. [39, 40, 49]
- **Ways out exist, but scale is the problem.** Holding health's share of GDP flat would need spending growth about 1.3 points lower every year, roughly $0.9 trillion less by 2034 (illustrative). Site-neutral payment, Medicare Advantage payment fixes and hospital price caps have credible evidence; drug negotiation, ACOs and PBM reform are each below half a percent of spending so far.

Implications

- **Policymakers.** The levers big enough to matter target prices and market power, payment rules that reward volume or coding, and administrative fragmentation. Demand-side cost sharing is largely exhausted: 34% of covered workers already face a deductible of $2,000 or more. [6]
- **Employers and other purchasers.** Evidence is strongest for reference-based hospital pricing, as in Oregon's and Montana's public plans, and weakest for higher deductibles, which reduce care indiscriminately, and for price transparency alone, which has not changed shopping. Tiered and narrow networks are used by a minority of firms with 50 or more workers (15% and 8%), but we did not retrieve outcome evidence on them. [6, 23, 30]
- **Households (system-level observation, not advice).** Premiums, deductibles and provider prices are set mostly by market structure and plan design outside individual control. Individual choices rarely bend the national curve.

Risks

- Modeled savings often exceed realized savings. An independent evaluation of the ACO REACH model, for example, found incentive payments offset the gross spending reduction for high-needs ACOs. [31]
- Spending can fall because people lose coverage, which is not lower cost. CMS expects growth to slow in 2027 to 2028 partly on lower Medicaid enrollment. [12]
- Price cuts to providers carry access and stability tradeoffs, especially for rural and safety-net hospitals. Oregon exempted them. [30]

Confidence: moderate Data on levels and prices are strong. Driver shares and savings estimates are model-dependent and come from different periods and definitions, so they cannot be added together.

1. Question, data and method

Question. Which parts of the health care system pushed spending and consumer costs up over roughly 40 years, how large is each, and what could slow the growth? "Customers" is read broadly: patients, employers, taxpayers and other payers.

Agents and stages. The Trend Scout framed the question, the Planner set hypotheses (prices, intensity, insurance design, market power, administration, demographics) and the Literature Surveyor grounded them in about 40 retrieved sources. The Quant and Modeling agents parsed CMS National Health Expenditure tables (1960 to 2024) and BLS, BEA and Census series, ran a growth-accounting decomposition and computed scenarios. The Brief Writer and Paper Author drafted both versions from one evidence base. The Fact-Checker and Red-Team, Bias Auditor and Ethics agents reviewed it.

Growth accounting. For personal health care spending, we split log growth between two years into four parts: (1) the GDP deflator, or economy-wide inflation; (2) the CMS personal health care price index relative to that deflator; (3) population; and (4) real spending per person, meaning CMS real dollars divided by population. Part 4 is a catch-all for use, intensity, service mix, technology and aging, because price indexes here do not separate quality change. The 1980 to 2024 window is 44 years; the CPI comparisons cover 1985 to 2025 (40 years).

> **How to read confidence labels.** **High**: multiple independent sources or randomized evidence agree. **Moderate**: consistent direction, but size varies by method. **Low**: soft estimates, contested definitions or thin evidence. Shares of growth from different studies use different years and definitions and are not additive.

2. The 40-year picture

National health expenditure (NHE) grew 7.2% to $5.3 trillion in 2024. Medicare paid $1.12 trillion (21%), Medicaid $932 billion (18%), private insurance $1.64 trillion (31%) and patients $557 billion (11%). Hospitals ($1.63 trillion) and physician and clinical services ($1.11 trillion) together account for just over half. [2] In 1980

spending was $253 billion, or $1,099 per person and 8.9% of GDP; the 2024 figures are 21 times the total and 14 times the per-person amount. [1]

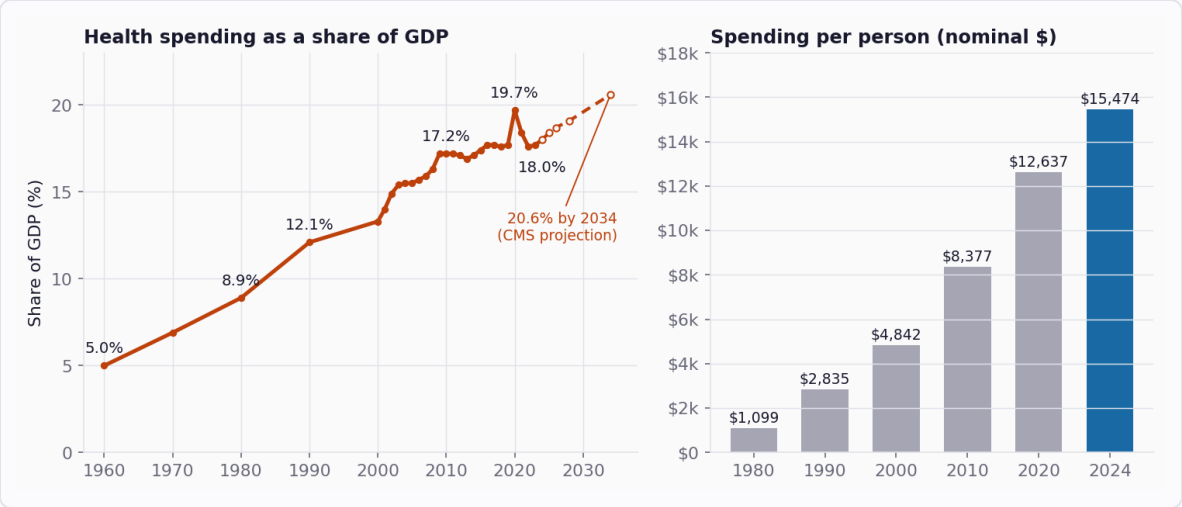


Figure 1. Health spending as a share of GDP (with CMS projections for 2025 to 2034) and nominal spending per person. Sources: CMS NHE Table 1; CMS 2025 to 34 projections. [1, 4]

Every major category outgrew the economy, but not equally. Prescription drugs grew 39-fold and home health 71-fold, while hospital care grew 16-fold against 10-fold for GDP. Hospitals' share of spending fell from 40% to 31% as drugs (4.7% to 8.8%) and overhead rose. The net cost of health insurance plus government administration grew from 4.7% to 7.0% of the total. [1]

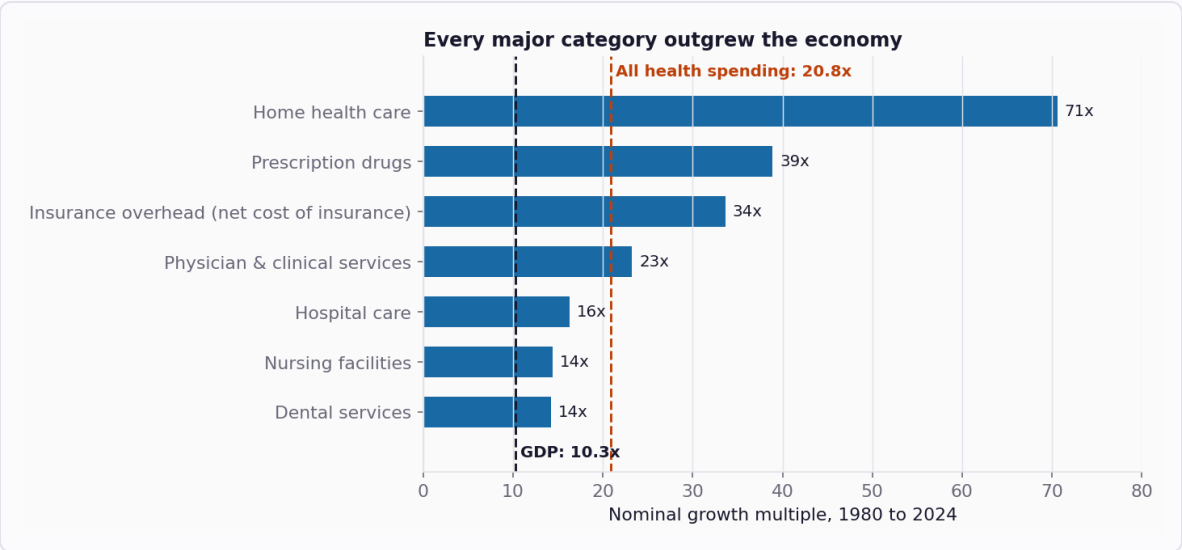


Figure 2. Nominal growth multiple by category, 1980 to 2024, against GDP (dashed) and total health spending. Source: CMS NHE Table 2; PRISM calculation. [1]

Growth accounting: where the growth came from

Personal health care spending grew 21-fold in nominal terms from 1980 to 2024. About 3.2-fold of the price change was economy-wide inflation, and medical prices added a further 1.45-fold. Population grew 1.5-fold, and real spending per person roughly tripled (3.1-fold). In shares of log growth, that is 38% general inflation, 12% medical-specific price growth, 13% population and 37% real use and intensity per person. [1, 5]

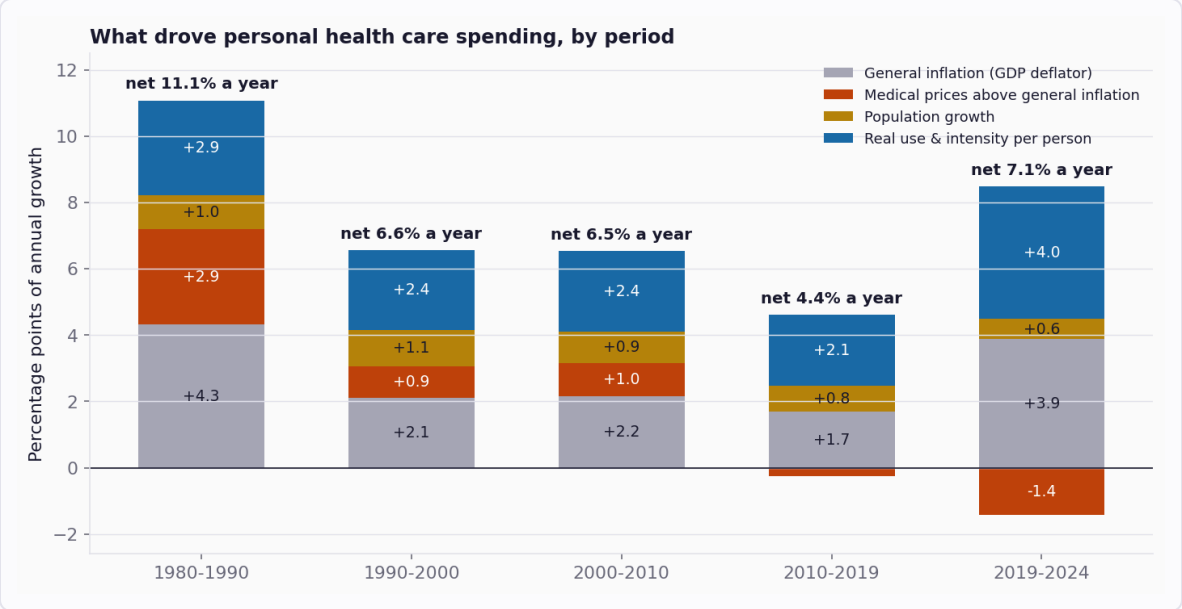


Figure 3. Components of annual growth in personal health care spending, percentage points, by period. Contributions are allocated by each component's share of log growth and scaled to the compound annual rate. Sources: CMS NHE Tables 1 and 23; BEA deflator via FRED; PRISM calculation. [1, 5]

Period	Nominal spending	General inflation	Medical price vs. inflation	Population
1980 to 1990	11.1%	4.2%	+2.8%	1.0%
1990 to 2000	6.6%	2.1%	+0.9%	1.1%
2000 to 2010	6.6%	2.1%	+1.0%	0.9%

Period	Nominal spending	General inflation	Medical price vs. inflation	Population
2010 to 2019	4.4%	1.7%	-0.2%	0.8%
2019 to 2024	7.1%	3.8%	-1.4%	0.6%
1980 to 2024	**7.2%**	**2.7%**	**+0.9%**	**0.9%**

Annual growth rates by component, compounded. Components multiply rather than add, so rows do not PRISM calculation from CMS NHE Tables 1 and 23 and BEA GDP deflator via FRED. [1, 5]

What this says. The 1980s were the price decade: medical prices ran 2.8 points a year above inflation. After the 1990s the price gap narrowed and turned negative in the CMS index, while real per-person use kept growing at 2 to 4% a year. CMS attributes 2024's growth mainly to use and intensity, with medical prices up about 2.5%. [3] That does not contradict the finding that prices explain the international gap: prices set the *level*, and intensity has driven recent *growth*. Three cautions apply. The CMS price index measures a fixed basket, so new and more intensive treatments show up as "real use". Its hospital component rose 5.4-fold from 1980 to 2024, against 15.7-fold for the BLS hospital services CPI, and the two are constructed differently; we could not reconcile them, and if the CPI is closer to the truth, prices explain more of past growth and "real use" less. And the 2019-to-2024 row spans the COVID-19 shock: utilization collapsed in 2020, rebounded sharply in 2021 and 2022, and hospital labor costs spiked from travel-nursing and staffing shortages, so that period's 4.0% real-use figure is noisier than the others and should not be read as a new steady-state growth rate. [1, 5]

Projections deserve skepticism. CMS's 2025 to 2034 projection assumes real per-person growth of about 2.8% a year, above the 1.7% observed from 2009 to 2023. Earlier CMS projections repeatedly overshot: in 1992 they showed 32% of GDP by 2030, and in 2010 they showed more than 19.1% by 2019. [11] The 2034 figures are a central case, not a certainty.

3. Coverage and benefits: who has been covered, and with what

Two things changed alongside the price and spending trends in Sections 1 and 2: how many people had insurance and through what channel, and what a standard policy actually paid for. Both shifted in ways that raised what insurers and taxpayers spend, even before any change in medical prices.

3.1 The long arc: from a private, employer-based system toward a five-part mix

Employer coverage is a byproduct of a 1940s wage freeze. Wartime wage controls did not limit fringe benefits, so employers competed for workers by offering health insurance instead of pay, and a 1943 War Labor Board ruling, later written into the tax code in 1954, confirmed those premiums were tax-free to the worker. By 1960, a Blue Cross/Blue Shield-dominated private market covered about 70% of Americans. Medicare and Medicaid, enacted in 1965, added coverage for people 65 and older and for many low-income people, layering public programs on top of an employer-centered private system rather than replacing it. [39]

The Centers for Disease Control's National Health Interview Survey has tracked coverage type since the late 1960s, and its longest consistent series, for the population under 65, shows the shape of the next forty years: private coverage held near 79% through 1980, then eroded, driven mainly by a decline in employer coverage from 71% in 1980 to 62% by 2007; Medicaid's share nearly doubled in the 1990s in particular; and the uninsured share drifted up from 12% in 1980 to the upper teens by the 1990s, where it stayed until the Affordable Care Act. [39]

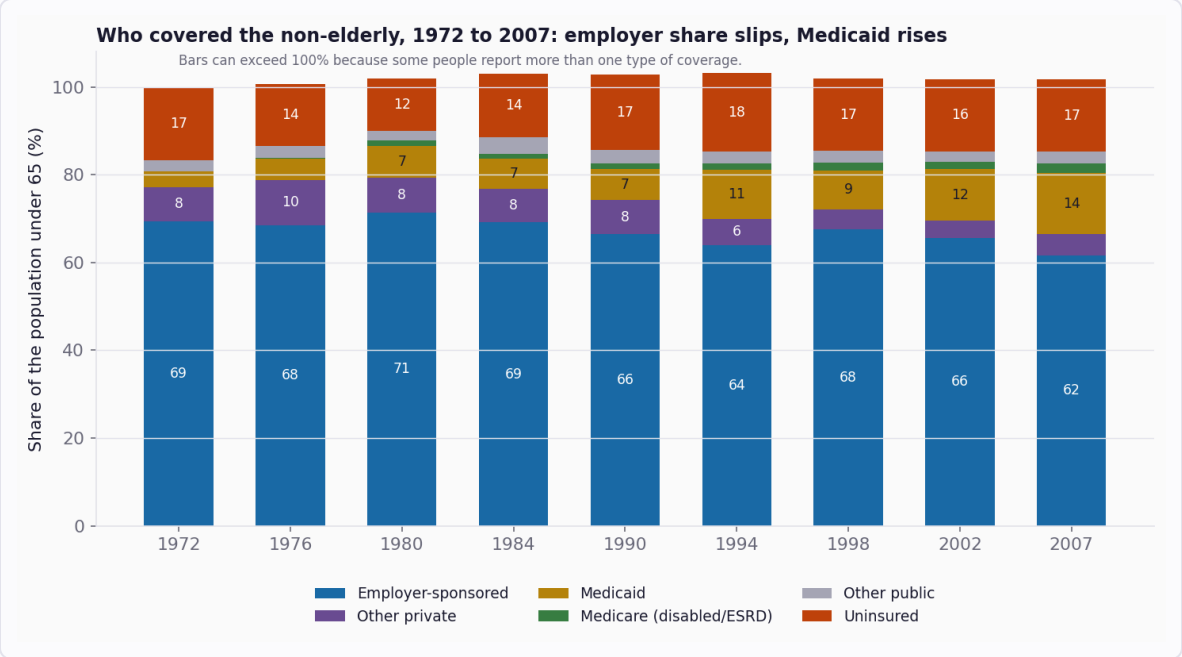


Figure 10. Health insurance coverage of the under-65 population, selected years 1972 to 2007. Bars can total slightly more than 100% because some people report more than one type of coverage. Source: CDC/NCHS National Health Interview Survey, historical series. [39]

Two inflection points explain most of the swings inside that chart. Medicaid's share jumped after the mid-1990s partly because eligibility was extended to more children and pregnant women through the 1980s, and partly because the survey itself started asking about Medicaid directly in 1978 rather than inferring it, a change that raises measured Medicaid coverage on its own. [39] The uninsured share rose fastest during the 1980-1990 recession-era decade, when employer coverage first began to erode, then plateaued near 16 to 18% for the next two decades. [39]

Since 2007. The Affordable Care Act's 2014 coverage expansions cut the non-elderly uninsured rate from about 16% in 2010 to a then-record low of 8.6% in early 2016, with an estimated 21.3 million people gaining coverage; a separate Urban Institute analysis using a different survey and calendar-year measure found the uninsured share falling from 17% in 2013 to 10% by 2016, with Medicaid expansion accounting for the majority (10.9 million) of the 18.5 million coverage gain it measured. [47] The two estimates differ because they use different surveys and time windows, but they agree on direction and rough scale. Coverage gains were sharpest in the 31 states (plus DC) that expanded Medicaid, where the uninsured rate roughly halved (15.3% to 7%) against a smaller drop in non-expansion states (19.8% to 13.7%). [47]

Coverage kept improving through the pandemic, when Congress paused Medicaid eligibility checks and richer marketplace subsidies pulled more people onto ACA plans, then gave some of that back after 2023 as the Medicaid "unwinding" resumed normal eligibility checks. By 2024 and 2025, the overall uninsured rate had settled at a historic low of about 8.2 to 8.3% of all ages (27 to 28 million people), with working-age adults at 11.6%, children at 5.1 to 5.6%, and people 65 and older at just 0.7%, since Medicare covers nearly everyone in that group. [40] Combining the CDC's age-group breakdowns, the under-65 uninsured rate was about 11% in 2021 and had fallen to roughly 10% by 2024 and 2025 (PRISM calculation, weighting reported child and working-age shares by population). [40] Coverage bought on the ACA marketplaces, which barely existed in the chart above, reached 6.3% of the under-65 population in 2025, up from 4.3% in 2021. [40] In the separate Census survey of full-year coverage, which counts differently and covers all ages, 92.1% of Americans had some coverage at some point in 2025: employer plans were the largest single source at 53.5%, followed by Medicare (20.1%), Medicaid (17.1%), directly purchased plans including marketplace coverage (10.5%), and military coverage (4.0%). [41]

Two of the levers behind that progress are now working in the other direction. Congress's 2025 reconciliation law (the One Big Beautiful Bill Act) adds Medicaid work-reporting requirements starting in 2027 and more frequent eligibility checks. CBO's estimates moved more than once as the bill changed in Congress; for the law as enacted, KFF's allocation of CBO's current estimates puts the total effect at about 10 million more uninsured by 2034 (Medicaid provisions 7.5 million, of which work requirements account for more than half; ACA marketplace provisions 2.1 million; other provisions and interactions 0.4 million). [51] Separately, the enhanced ACA premium tax credits that had capped many enrollees' payments expired at the end of 2025; combined with that expiration, CBO's estimates imply roughly 14 million more uninsured by 2034, which is expected to push marketplace enrollment down as healthier, price-sensitive enrollees leave first (Section 6). [51, 36] Whether the uninsured rate keeps falling or reverses over the next decade now depends heavily on how those two policies play out and on whether Congress revisits the subsidies.

Confidence **high on the historical pattern and current levels**, **moderate on 2027 to 2034 projections, which are CBO estimates, not outcomes**

3.2 What changed inside the standard benefit package

Coverage got broader almost every decade, largely through federal mandates layered onto the existing private and public systems rather than through a single redesign. The main additions, roughly in order:

- **Continuation rights (1985).** COBRA required employers with 20 or more workers to let departing employees buy into the group plan, at their own cost, for up to 18 months (longer for some events). It did not make coverage cheaper, only available. [39]
- **Portability and a first, narrow parity rule (1996).** HIPAA limited how long group plans could exclude a new enrollee's pre-existing condition and set portability standards between jobs. The same year's Mental Health Parity Act barred group plans from setting lower lifetime and annual dollar limits on mental health benefits than on medical care, but it did not require plans to cover mental health at all, and it left copays, deductibles, and visit limits untouched. [39, 43]
- **Children's coverage (1997).** CHIP extended subsidized coverage to children in families with incomes above Medicaid's cutoff but still moderate, financed through federal-state matching similar to Medicaid's. [39]
- **Consumer-directed accounts (2003).** The Medicare Modernization Act created health savings accounts, tax-advantaged accounts usable only alongside a high-deductible plan. HSA-linked enrollment grew from about 1 million people in 2005 to over 20 million policyholders and dependents by 2016, and by 2025 33% of covered workers were in an HDHP with a savings option, up from single digits in the mid-2000s. [6, 44] This is a benefit-design change more than a benefit expansion: it pairs a tax-favored account with higher up-front cost sharing, and it is the same mechanism behind the rising deductibles discussed in Section 4.5 below.
- **A real Medicare drug benefit (2006).** The same 2003 law created Medicare Part D, the first broad outpatient prescription drug coverage in the program's history; the 2022 Inflation Reduction Act later added a $2,000 out-of-pocket cap (2025) and price negotiation (Section 7.2). [39, 32]
- **Full mental health and substance use parity (2008).** The Wellstone-Domenici Act extended the 1996 law's dollar-limit parity to copays, deductibles, visit limits,

and prior-authorization practices, and added substance use disorder treatment, for group plans of 50 or more workers; the ACA later extended parity to individual and small-group plans. Regulators estimated it affected 62 million Americans, including about 23 million who met criteria for a substance use disorder. It still does not require a plan to cover mental health or addiction treatment, only to cover it on equal terms if it does, and more than 90% of commercial plans already did. [43]

- **The ACA's benefit floor (2010 to 2014).** Effective September 2010: dependents could stay on a parent's plan to age 26 (an estimated 3 million young adults gained coverage by the end of 2011), lifetime dollar limits on essential benefits were banned, and insurers could no longer exclude children under 19 for a pre-existing condition. [46] Effective 2014 for non-grandfathered individual and small-group plans: guaranteed issue at any age regardless of health status, a ban on annual as well as lifetime limits, and a required floor of ten essential health benefit categories, including maternity care, mental health and substance use treatment, and prescription drugs, benefits that many individual-market plans had not covered before. The same plans must cover preventive services rated "A" or "B" by the U.S. Preventive Services Task Force with no copay or deductible. [39, 42]
- **A ban on most surprise bills (2022).** The No Surprises Act barred out-of-network balance billing for emergency care, air-ambulance transport, and ancillary providers (anesthesiologists, radiologists, pathologists, assistant surgeons) at in-network facilities, limiting the patient to in-network cost-sharing and routing any remaining dispute to insurer-provider arbitration. It left two notable gaps: ground ambulances, which can still bill out-of-network in most states absent separate state protections, and non-emergency care at facilities the law does not cover, such as freestanding urgent-care centers not licensed for emergency services and addiction-treatment facilities. [52]
- **Two 2025 rulings that cut opposite ways.** In June 2025 the Supreme Court, in *Kennedy v. Braidwood Management*, upheld the constitutionality of the Task Force's structure by a 6-3 vote, preserving the no-cost preventive-care mandate nationally; the Court did not rule on parallel claims about the vaccine and HIV-prevention advisory bodies. [50] The same year's reconciliation law added Medicaid

work-reporting requirements and more frequent eligibility redeterminations (Section 3.1), which raise the bar for keeping coverage rather than changing what a covered person's plan pays for. [49]

The net effect is that the floor under what a "covered" person receives is much higher than it was in 1980: routine screenings, mental health and maternity care, and a young adult's dependent status are now standard in ways they were not, while public coverage now reaches a broader income band. That floor is part of why real per-person spending grew (Section 2): a plan that must cover maternity care, mental health treatment, and preventive visits at no charge costs more to underwrite than one that does not, even before any change in prices.

3.3 Medicare's own benefit package kept expanding

Medicare followed a similar path from a narrower to a broader benefit set. Part A and Part B (hospital and physician coverage) date to 1965; Medicare+Choice, a private-plan alternative later renamed Medicare Advantage, was created in 1997; and Part D added drug coverage in 2006. Medicare Advantage plans are not required to offer extra benefits, but most compete for enrollees by bundling in dental, vision, hearing, or fitness benefits that traditional Medicare does not cover, funded in part by the higher payments MedPAC has documented (Section 4.6). Enrollment in these private plans grew from 19% of eligible beneficiaries in 2007 to 54% in 2025, and CBO projects 64% by 2034, meaning most of the growth in what Medicare "covers" for a given beneficiary in the last two decades has come through this privately administered, supplemental-benefit-rich channel rather than through changes to the traditional program. [48]

3.4 The push and pull: broader benefits, deeper cost sharing

The benefit mandates above and the cost-sharing trend covered in Section 4.5 ran in opposite directions at the same time. Plans had to cover more, with fewer dollar limits and no medical underwriting, while the typical enrollee took on a larger deductible. Both raised what insurers had to price for, for different reasons: mandates raised the floor of what any plan must pay, while high-deductible designs shifted the first dollars of that spending back onto the patient. The net effect on family budgets shows up in

Section 6: premiums and deductibles both rose, even as the coverage behind them became more comprehensive and harder to be denied.

3.5 Timeline



Figure 11. Milestones that changed who is covered and what coverage includes, 1943 to 2026. Sources given in Section 3.2. [39, 42, 43, 44, 45, 49, 50]

Reading this timeline. Blue markers changed the rules for private, mostly employer, coverage; gold markers changed a public program; green markers added or strengthened a required benefit. Several dates load more than one kind of change,

which is itself a sign of how tangled the system had become by the 2000s. Sources for each event are given in the paragraphs above. [39, 42, 43, 44, 45, 49, 50]

3.6 What broader coverage still leaves on the table: underinsurance and medical debt

Being insured is not the same as being protected from cost. As deductibles and out-of-pocket maximums rose (Section 4.5), a growing share of the "covered" population became underinsured: enrolled in a plan but exposed to costs large enough to strain a household budget. Estimates of the resulting medical debt vary widely by definition and data source, which is itself informative about how hard the problem is to measure precisely. A 2024 KFF analysis of national survey data put total U.S. medical debt at roughly $220 billion, with about 6% of adults (around 14 million people) owing more than $1,000 and about 1% owing more than $10,000. [53] A broader 2022 KFF Health News and NPR investigation, using a wider definition that includes credit-card balances, loans from family, and payment plans taken on specifically to cover medical bills, estimated that about 100 million adults, or 41% of the adult population, carried some form of health care debt. [54] The two figures are not inconsistent; they measure different things, and the narrower, collections-style estimate and the broader, self-reported one bracket a range rather than contradict each other. Confidence **high that debt from medical bills is widespread**, **low on a single precise national total**

3.7 Self-funded employer plans and ERISA preemption

Most of the private coverage described above sits inside a structural fact that shapes whether state consumer-protection and benefit-mandate laws reach it. Under the Employee Retirement Income Security Act (ERISA) of 1974, an employer that self-funds its health plan, bearing the claims risk itself rather than buying an insurance policy, is largely exempt from state insurance regulation, including many state benefit mandates and consumer protections. In the 2024 KFF Employer Health Benefits Survey, 63% of covered workers were in a self-funded plan, rising to 79% at firms with 200 or more workers and 20% at smaller firms. [55] That share has grown for decades as large employers moved away from fully insured plans, and it means state benefit mandates, state-level surprise-billing protections that preceded the federal No

Surprises Act, and state public-option or reinsurance programs reach a shrinking share of the privately insured workforce. Federal rules, including the No Surprises Act and the ACA's benefit floor, apply regardless of funding arrangement, which is part of why recent consumer protections have been written at the federal rather than the state level.

4. The drivers, one by one

The evidence supports several drivers that operate at the same time and reinforce each other. They are grouped below from the largest and best documented to the smallest and softest. Each subsection states the mechanism, the evidence, the counter-evidence and a confidence rating.

4.1 Medical technology and rising intensity of care

Mechanism. New drugs, devices, imaging and procedures expand what medicine can do, and insurance and rising incomes make people willing and able to use them. Newhouse's line of work treats technology as the main long-run engine, with income and insurance as enabling conditions. Smith, Newhouse and Freeland estimate technology explains 27 to 48% of spending growth since 1960, a smaller share than earlier estimates, and note that income growth works largely through governments and employers buying on behalf of pools of consumers. [15] They also find insurance contributes less through more generous coverage and more through changes in how providers are paid. [15]

Evidence in our data. Real spending per person, our catch-all for use, intensity, technology and aging, grew about 2.6% a year over 44 years and accelerated after 2019. A decomposition of 1996 to 2013 found service price and intensity accounted for more than half of the $933.5 billion increase. [13] CMS attributes 2024 growth mainly to use and intensity. [3]

Counter-evidence. These are residual-based estimates. "Technology" is what is left after measured factors, so it can absorb mismeasured prices and demographic change. Some new treatments also deliver large health gains, so higher spending is not automatically waste. Confidence **moderate on direction, low on the exact share**

4.2 High prices for hospitals, drugs and labor

The international comparison. The U.S. spends about twice as much as other high-income countries, yet utilization is broadly similar. Prices of labor and goods, including drugs, and administrative costs appear to be the main reasons. Administration was 8% of U.S. spending against 1 to 3% elsewhere; U.S. per-person drug spending was $1,443 against $466 to $939; and generalist physicians earned $218,173 against $86,607 to $154,126. [14] In 2024 U.S. per-person health consumption spending was $14,775, almost $5,000 above the next-highest peer, Switzerland, and about double the peer average of $7,860. [9]

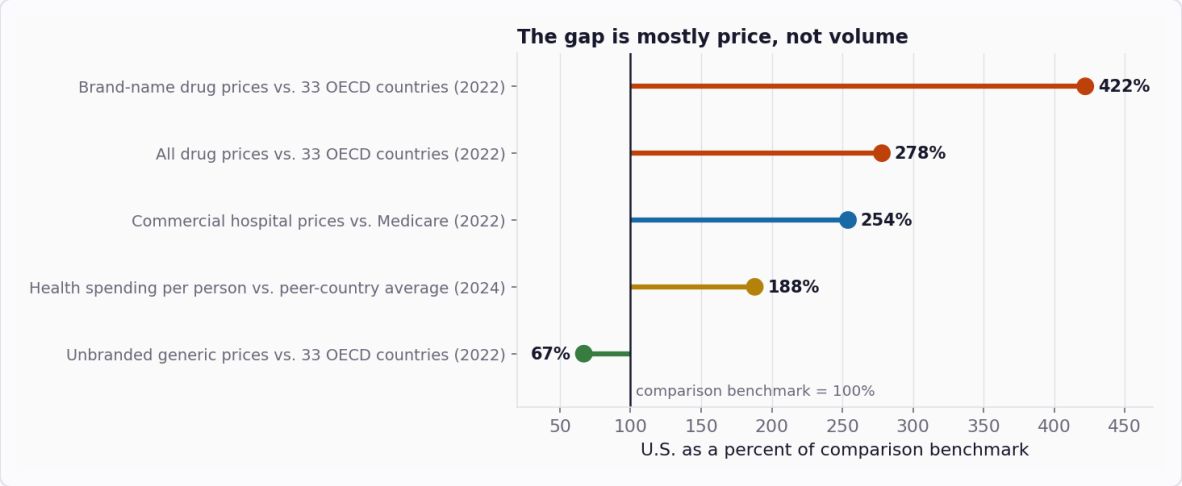


Figure 4. U.S. prices and spending relative to benchmarks (benchmark = 100%). Drug prices: RAND/ASPE 2022 data; hospital prices: RAND Round 5.1; per-person spending: Peterson-KFF 2024 (14,775 vs. 7,860 dollars). [9, 18, 19]

Prices over time. Consumer price data show the price divergence is concentrated in hospitals. From 1985 to 2025 the hospital services index rose 9.8-fold (5.9% a year). Professional services rose 3.9-fold and medical goods 3.6-fold, roughly in line with hourly earnings (3.6-fold), while all-items CPI rose 3.0-fold. [5]

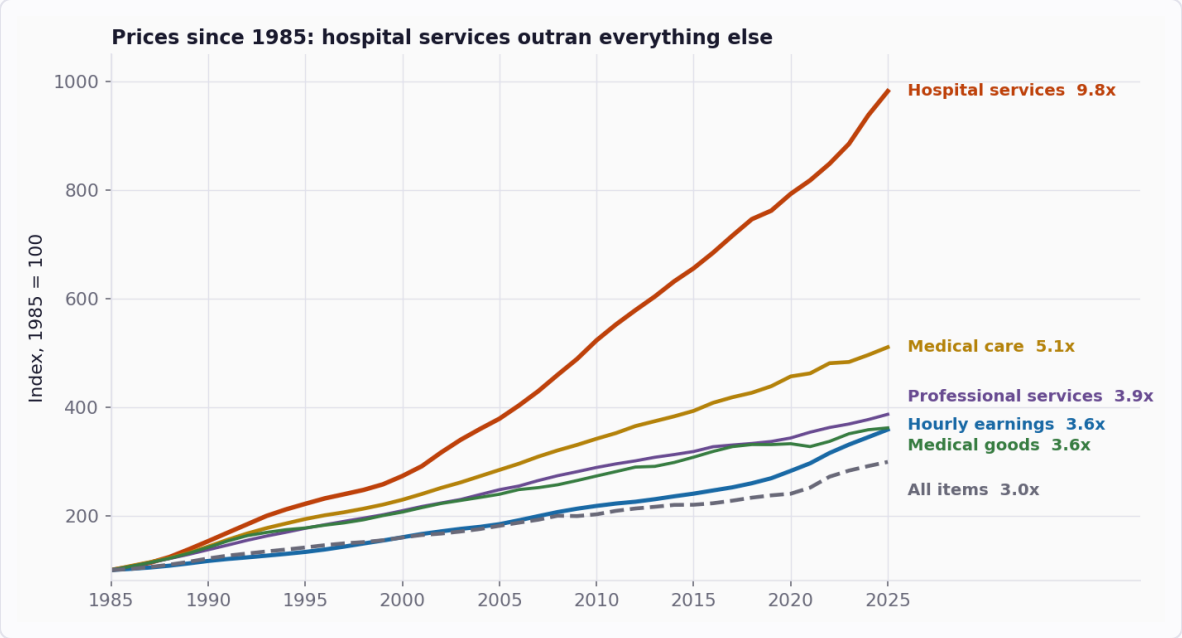


Figure 5. Price indexes, 1985 to 2025 (annual averages, 1985 = 100). Sources: BLS CPI and average hourly earnings via FRED; PRISM calculation. [5]

Category	1980	2024	Multiple
Hospital care	22.3	121.3	5.4x
Physician and clinical services	36.8	108.0	2.9x
Prescription drugs	15.1	99.1	6.6x
Nursing care facilities	23.5	128.1	5.5x
Home health care	29.4	121.7	4.1x
GDP deflator (economy-wide)	39.4	125.4	**3.2x**

CMS price indexes by category, 1980 to 2024 (2017 = 100), against the economy-wide GDP deflator. Source: CMS NHE Table 23; BEA via FRED. [1, 5] Index construction differs from the CPI, and drug indexes have limited ability to reflect new products.

Two features stand out. Physician prices in the CMS index grew more slowly than inflation (2.9-fold against 3.2-fold), possibly reflecting the pull of Medicare's fee schedule, though we did not test that. Hospital prices ran well ahead of inflation. Prices relative to Medicare have not risen recently, but they remain high: state-level median commercial prices were 254% of Medicare in 2018, 246% in 2020 and 253% in 2022 across three RAND rounds. [18] Confidence **high on the level gap, moderate on how much of historical growth it explains**

4.3 Provider consolidation and market power

Mechanism. When one hospital system dominates a market, insurers cannot credibly threaten to exclude it, so it can negotiate higher prices. Hospitals also buy physician practices, which lets them bill higher facility rates and negotiate jointly.

Evidence. In claims covering 28% of people with employer coverage, prices at monopoly hospitals were 12% higher than in markets with four or more rivals. Concentrated *insurer* markets showed the opposite: a 10-point increase in insurers' collective share was associated with 7% lower hospital prices. [16] Half the variation in private spending across regions came from price and half from quantity; in Medicare, where prices are set administratively, regional variation is almost entirely quantity. [16] A one-standard-deviation increase in the market share of hospitals owning physician practices was associated with 2 to 3% higher prices and spending. [17] RAND finds commercial hospital prices ranged from under 170% of Medicare in Arkansas to over 300% in eight states, and that within states the gap between 25th- and 75th-percentile hospitals implies a potential 45% cut in hospital spending. [18]

How big, in dollars. A wider literature on specific mergers puts a range on the effect. Hospitals with no nearby competitor charge about 12% more than those with three or more rivals, and a typical merger between hospitals within five miles of each other raises prices by about 6%, with the increase persisting for at least two years. [58] Mergers that create a near-monopoly can raise prices far more: an early estimate put the ceiling at 53%, and specific, closely studied mergers have been associated with price increases of 44% to 65%. [58] When hospitals acquire physician practices, one study found average price increases of about 14% and spending increases of about 4.9%; a separate per-merger estimate found average increases of $521 in hospital prices and $579 in spending for the privately insured, alongside a $638 reduction in affected workers' wages. [58] These are estimates from specific, well-studied transactions, not a national total, and should not be summed into one aggregate figure.

Counter-evidence. Integration can also improve coordination, which is why accountable care encourages it. [17] And insurers wield their own market power, which cuts both ways for premiums. Confidence **high on price effects, low on the aggregate contribution to 40-year growth**

4.4 Drug pricing, exclusivity and pharmacy benefit managers

Evidence. In 2022, U.S. manufacturer prices for all drugs were 278% of prices in 33 other high-income countries, and 422% for brand-name originators. After adjusting for U.S. rebates the brand figure was still 308% to 322%. Unbranded generics, about 90% of U.S. prescriptions by volume, were cheaper in the U.S. at 67% of foreign prices. The gap has been widening. [19] Retail prescription drug spending rose from 4.7% to 8.8% of NHE from 1980 to 2024. [1] The largest PBMs, which negotiate rebates and manage formularies, are a handful of vertically integrated firms that FTC research reportedly shows handle about four-fifths of prescriptions. [33]

Counter-evidence. Manufacturers and some economists argue that high brand prices fund innovation, and price comparisons also depend on product mix. GLP-1 obesity drugs illustrate the tension: 43% of employers with 5,000 or more workers now cover them for weight loss, and 66% of those firms say the effect on their drug spending is significant. [6] Confidence **high on price levels**, **moderate on causes**

4.5 Insurance design, third-party payment and the tax exclusion

Mechanism. When someone else pays, patients and clinicians weigh price less. The employer tax exclusion also makes compensation in the form of premiums cheaper than wages.

Evidence. Patients' own payments fell from 52% of consumption spending in 1960 to 24% in 1980 and 11% in 2024, while Medicare grew from zero to 22% and Medicaid to 18%. [1] In the RAND Health Insurance Experiment, families facing 95% coinsurance spent about 30% less than families with free care, almost entirely by using less care, and the reductions fell about equally on highly effective and less effective services. The sickest and poorest did worse on some outcomes without free care. [22] A natural experiment at a large employer that moved workers from free care to a high-deductible plan found spending fell 12 to 14%, entirely from reduced quantity, with no sign that workers learned to shop for lower prices. [23] CBO has said the open-ended employer tax exclusion, which forgoes about $250 billion in revenue a year, increases spending by encouraging more comprehensive coverage and benefits higher earners most. [35] (The figure is from an older estimate; current amounts are likely larger.)

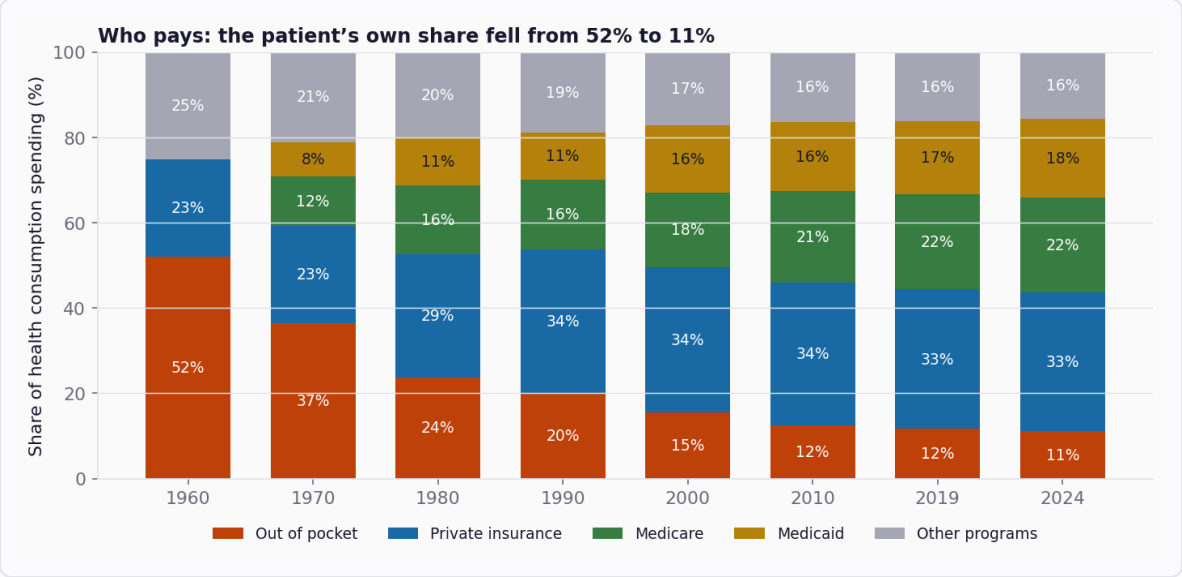


Figure 6. Share of health consumption spending by payer. Out-of-pocket payments fell from 52% to 11% as Medicare, Medicaid and private insurance grew. Source: CMS NHE Table 3; PRISM calculation. [1]

Counter-evidence. The RAND experiment excluded people over 64, and critics argue attrition in the cost-sharing arms weakens its conclusions. [22] Smith and colleagues find generosity of coverage explains less of growth than technology does. [15]

Confidence **high that price insensitivity raises use, moderate on its share of long-run growth**

4.6 Payment design and Medicare Advantage

Mechanism. Payment rules shape what gets provided and where. Medicare's inpatient prospective payment system began in 1983, and its physician fee schedule (RBRVS) phased in from 1992; both replaced cost- or charge-based payment, though payment still scales with the number of cases or services. [37] Payment differences by site of care let hospitals bill more for the same service: MedPAC found chemotherapy billed through hospitals rose from 35.2% in 2012 to 51.9% in 2021. [28]

Evidence. MedPAC estimates Medicare will spend about $76 billion, or 14%, more on Medicare Advantage enrollees in 2026 than on similar people in traditional Medicare, down from $84 billion in 2025 after a new risk model. Favorable selection accounts for roughly 11 points and coding intensity roughly 4. Medicare Advantage covers more than half of beneficiaries. [27] CBO scores paying site-neutral rates for most hospital outpatient services at $156.9 billion in savings over 2025 to 2034. [28]

Counter-evidence. The insurer-backed Better Medicare Alliance disputes MedPAC's methods, saying they ignore the program's value to beneficiaries. [27] Safeguards for rural and safety-net hospitals matter in any site-neutral design. Confidence **high that the distortions exist, moderate on aggregate effect on the 40-year trend**

4.7 Administrative complexity

Evidence. In CMS accounts, the net cost of health insurance plus government administration was $372 billion in 2024, 7.0% of NHE, up from 4.7% in 1980. [1] A broader estimate that adds hospital, physician and long-term-care administration puts U.S. administration at $812 billion, or 34.2% of spending in 2017, against 17.0% in Canada. Insurer overhead was $844 per person against $146. Most of the 3.2-point rise in administration's share since 1999 came from private insurers' overhead, especially in Medicare and Medicaid managed-care plans. [20]

Counter-evidence. Definitions matter. Accounting categories differ between the two countries, and the broad estimate excludes dentists, pharmacies and some other providers. [20] The study was publicized through Physicians for a National Health Program, a single-payer advocacy group. Analysts also disagree on how much administration a single payer would cut: Mercatus notes CBO assumed lower payer administrative spending than other analyses did. [34] Confidence **moderate: direction is clear, the size is definition-dependent**

4.8 Demographics and disease burden

Evidence. In 1996 to 2013, population growth accounted for 23.1% of the spending increase and aging for 11.6%, while changes in disease prevalence or incidence were negatively associated with spending. [13] In our accounting, population growth explains 13% of 1980 to 2024 growth. Obesity adds to costs: one study estimated it raised annual medical spending by $2,505 per adult with obesity and $260.6 billion in aggregate in 2016. [25] Aging will matter more going forward: CMS projects Medicare, with 22% of the population and 26% of spending by 2034, to grow fastest at 7.7% a year as baby boomers age in. [4, 38]

Counter-evidence. Aging is a modest historical driver but a rising future one. Attribution studies cover different windows, so the 12% aging share should not be projected forward. Confidence **high that aging was not the main historical driver, moderate on the future**

4.9 Labor costs and Baumol's cost disease

Labor-intensive services with slow productivity growth get more expensive relative to the economy because wages must keep pace with sectors where productivity rises. Studies applying Hartwig's test find a significant effect on health spending growth, while skeptics argue high-tech acute care does not fit the model's assumptions. [26] U.S. clinician pay is also high by international standards. [14] Residency slots for Medicare payment purposes were capped in 1997, though hospitals kept adding residents beyond the cap. [37] We did not retrieve causal estimates of how much workforce supply limits raised prices. Confidence **low to moderate**

4.10 Waste, fraud and defensive medicine

One widely cited analysis estimated waste at $760 to $935 billion a year, about 25% of spending, across overtreatment, failures of care delivery and coordination, pricing failures and fraud, with $191 to $286 billion potentially recoverable. [21] Two of its authors reported funding from Humana and the third reported employment with UPMC Health Plan, so the estimate deserves the same scrutiny as advocacy-group figures. [21] Malpractice and defensive medicine total an estimated $55.6 billion (2.4% of spending in 2008), of which defensive medicine is about $45.6 billion, and the authors concluded liability reform is unlikely to produce large savings. [24]
Confidence **low to moderate**

4.11 Private equity and corporate ownership of physician practices

Mechanism. Beyond hospital mergers, a newer form of consolidation has grown inside physician practices themselves. Private equity firms acquire practices, often roll several into a larger regional platform, and typically plan to resell within three to seven years at a profit, which can create pressure to raise revenue, increase referrals

within the platform, or substitute lower-cost clinicians, independent of any change in the underlying cost of care.

Evidence. Private equity acquisitions of physician practices rose sharply after 2013 and concentrated in a handful of specialties: in one tracking study of acquisitions through 2016, anesthesiology and multispecialty groups each accounted for about 19% of deals, emergency medicine 12%, family practice 11%, and dermatology 10%, with later industry reporting showing continued growth in gastroenterology, ophthalmology, dermatology, and orthopedics. [57] A 2023 to 2024 analysis found that in 28% of local physician markets studied, a single private-equity-owned firm controlled more than 30% of physicians in at least one specialty, and in some specialty-market combinations, over half of practicing physicians worked for one such firm. In markets concentrated to that degree, prices rose 18% in gastroenterology, 16% in obstetrics and gynecology, and 13% in dermatology; across ten specialties studied, prices rose 4% to 16% in eight of them following acquisition. [56]

Counter-evidence. Consolidation under any owner, private equity, hospital, or large independent group, can fund capital investment, electronic records, and after-hours coverage that small independent practices struggle to afford, and not every acquisition is followed by a measurable price increase. The research base is also younger and thinner than for hospital mergers, and most studies cover a handful of specialties and markets rather than a national sample. Confidence **moderate on price effects in the specialties studied**, **low on the national aggregate**

4.12 Physician and specialist compensation

Mechanism. The United States pays doctors more than other high-income countries pay theirs, and pays specialists substantially more than generalists within the U.S. Higher pay raises the price of physician services directly, and because many downstream prices (facility fees, imaging, procedures) are built around physician charges, it also raises prices elsewhere in the system.

Evidence. A 2011 Health Affairs study found that higher fees paid to physicians, not more frequent visits or procedures, explained most of the gap between U.S. and peer-country spending on physician services. [59] That gap has persisted: a 2023

international survey put average U.S. physician compensation at $352,000, against $273,000 in Canada, $160,000 in Germany, $122,000 in the United Kingdom, and $93,000 in France. [60] Within the United States, specialists also earn substantially more than primary care physicians: a 2024 survey put average primary care compensation at $281,000 against $398,000 for specialists, with orthopedics the highest-paid specialty at $543,000 and public health and preventive medicine the lowest among physicians at $257,000. [60] That gap is itself partly a policy artifact: Medicare's physician fee schedule, which many private payers reference, has long paid procedural specialties more per unit of work than primary care, shaping the pipeline of medical students into higher-paying fields.

Counter-evidence. International pay comparisons are sensitive to exchange rates, tax treatment, malpractice costs, and the length and cost of training, all of which run higher in the U.S. and are not pure "overpayment." Physician compensation is also only part of what a physician service bills for; facility and administrative overhead make up a large share of a typical charge. Confidence **high that U.S. physician pay is high by international standards**, **moderate on how much of the spending gap it alone explains**

4.13 Other narrower cost factors: 340B, certificate-of-need laws, and nonprofit tax exemptions

Three smaller, frequently debated factors do not individually move the 40-year total much but recur often enough in policy debate to note. **The 340B drug discount program** requires manufacturers to sell outpatient drugs at a discount to hospitals and clinics serving low-income populations; covered-entity purchases under the program grew from about $66 billion in 2023 to about $81 billion in 2024, and critics argue that some hospitals use the margin between the discounted acquisition price and what they charge insurers to cross-subsidize general operations rather than low-income care, a use the program's defenders say is consistent with its design. [61] **Certificate-of-need (CON) laws**, which require state approval before a hospital can add beds or equipment in roughly 35 states, are intended to prevent costly duplication, but a systematic review found the evidence on their net effect on costs mixed, and in the review's own cost-benefit estimate, found costs exceeding benefits by roughly 8% nationally, with

wide uncertainty around that figure. [62] **Nonprofit hospitals' tax exemption**, justified by the charity care and community benefit they provide in exchange, was worth an estimated $37.4 billion in federal, state, and local tax benefits in 2021; one advocacy group's "fair share" analysis found that 227 of 275 nonprofit hospital systems studied spent less on charity care and community investment than their tax exemption was worth, by a combined $18.4 billion, though nonprofit hospitals dispute how that benchmark is calculated. [63] Confidence **low to moderate** on all three as aggregate cost drivers

5. Driver scorecard

Driver	Best evidence on size	Confidence
Technology and rising intensity of care	27 to 48% of growth since 1960 [15]; price and intensity over 50% of 1996 to 2013 increase [13]; about 37% of 1980 to 2024 log growth is real per-person use (PRISM)	**Moderate**
High prices for hospitals, drugs and labor	Prices and administration, not utilization, explain the gap with peers [14]; commercial hospital prices 254% of Medicare [18]; drug prices 278% of peers [19]	**High** (level); **moderate** (growth)
Provider consolidation and market power	Monopoly hospitals +12% [16]; hospital-owned practices +2 to 3% per standard deviation [17]; specific mergers +6 to 65% depending on market power gained [58]	**High** (price effect); **low** (aggregate)
Insurance design and tax exclusion	Patient share 52% to 11% of spending [1]; cost sharing cuts spending 20 to 30% [22]; exclusion about $250 billion a year [35]	**High** (mechanism); **moderate** (share)
Payment design, site of care, Medicare Advantage	$76 billion (14%) Medicare Advantage gap in 2026 [27]; site-neutral about $157 billion over 10 years [28]	**High** (exists); **moderate** (aggregate)
Administrative complexity	7.0% of NHE (CMS definition) up to 34.2% (broad, contested) [1, 20]	**Moderate**

Driver	Best evidence on size	Confidence
Demographics and disease burden	Population 23.1%, aging 11.6% of 1996 to 2013 increase [13]; obesity $260.6 billion in 2016 [25]	**High** (past); **moderate** (future)
Labor costs (Baumol) and supply limits	Contested literature [26]; clinician pay well above peers [14]	**Low** to **moderate**
Waste, fraud, defensive medicine	Waste 25% (funder-disclosed) [21]; liability system 2.4% [24]	**Low** to **moderate**
Private equity and corporate ownership of physician practices	Over 30% specialty-market concentration in 28% of markets studied [56]; price increases 4 to 18% in concentrated specialty markets [56]	**Moderate** (price effect); **low** (aggregate)
Physician and specialist compensation	U.S. average $352,000 vs. $93,000 to $273,000 in peer countries [60]; U.S. specialists $398,000 vs. primary care $281,000 [60]	**High** (level); **moderate** (share of gap)
340B, certificate-of-need laws, nonprofit tax exemptions	340B purchases $81 billion in 2024 [61]; CON laws' net cost about 8% of affected spending [62]; hospital tax exemption $37.4 billion in 2021 [63]	**Low to moderate**

PRISM synthesis. Sizes come from different studies, periods and definitions and cannot be added. Numbers in brackets link to sources.

6. What customers actually pay

Family coverage through an employer averaged $26,993 a year in 2025 (single: $9,325), up 6% (5%) in a year. Workers paid $6,850 toward family coverage and $1,440 for single, about 26% and 16% of the premium. The average single deductible was $1,886, and 34% of covered workers had a deductible of $2,000 or more; average deductibles are up 43% over ten years. [6] The 1999 family premium was $5,791, so premiums have risen 4.7-fold since then. [6, 7] A 2025 analysis of KFF data found that from 1999 to 2024, family premiums rose 342% and workers' contributions 308%, while workers' earnings rose 119% and prices 64%. [8]

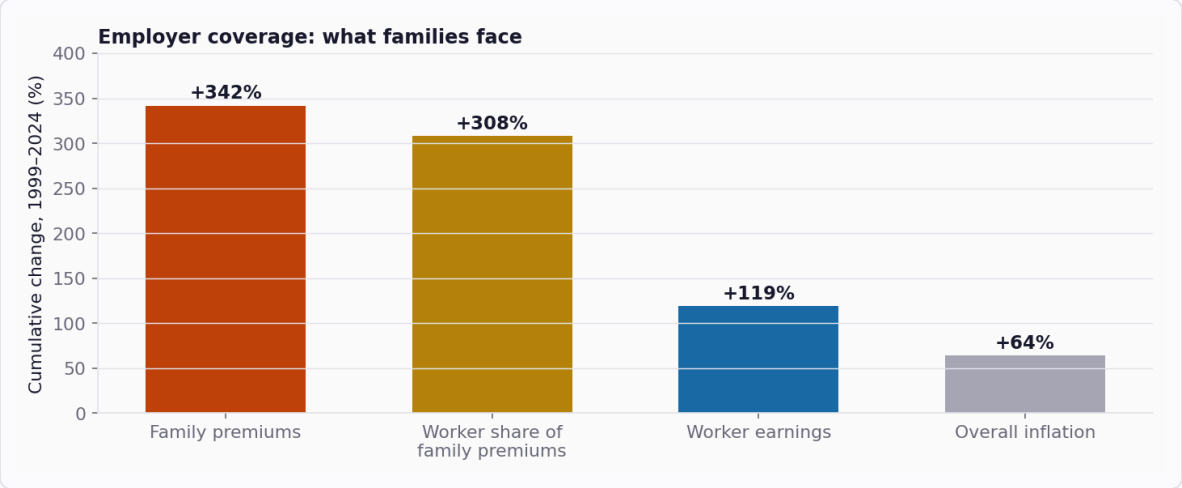


Figure 7. Cumulative change in employer family premiums, worker contributions, earnings and prices, 1999 to 2024. Source: 2025 research letter using KFF survey and BLS data. [8]

A nuance. Over the most recent five years, family premiums rose 26%, in line with wage growth (28.6%) and inflation (23.5%). KFF expects higher cost trends for 2026. [6] The squeeze on families is therefore a long-run pattern, and the last decade has been calmer than the first two.

Access and outcomes. In 2024, 17% of U.S. adults reported delaying or skipping care because of cost, and U.S. life expectancy (79.0 years) remained 3.7 years below the average of comparable countries. [10] Over the long run, real median household income rose about 41% between 1984 and 2025, while real personal health care spending per person roughly tripled between 1980 and 2024. [1, 5]

The 2026 marketplace shock. Enhanced ACA premium tax credits expired at the end of 2025. KFF measured a 58% rise in average monthly premium payments net of credits, from $113 to $178, less than its 114% projection because many enrollees switched to cheaper plans and high-premium enrollees left the market. [36] CMS projects the insured share to fall from 91.8% in 2024 to 90.5% in 2034. [4] Shifting who pays, or how many are covered, changes household exposure but not the underlying price and intensity drivers.

7. Ways out

A lever is worth pursuing if it targets a large driver, has evidence that it works, is big enough to matter and has manageable side effects. The levers are grouped by the driver they attack. Savings estimates are annual and expressed against 2024 national health spending of $5.28 trillion, except where noted.

7.1 Attack prices and market power

Site-neutral payment. Paying the same rate for a service whether it is delivered in a hospital outpatient department or a physician's office removes a reason to buy practices and bill facility fees. CBO scores site-neutral rates for most hospital outpatient services at $156.9 billion over 2025 to 2034 (about $16 billion a year on average), and a later CBO edition summarized by CRFB puts a similar policy at $180 billion over 2026 to 2035. [28] Concerns focus on rural and safety-net hospitals. [28]

Hospital price caps and reference pricing. Oregon capped what its state employee plan pays hospitals at 200% of Medicare (185% out of network) from late 2019 and exempted small, rural, critical-access and some sole-community hospitals. The cap saved more than $100 million in its first 27 months, about 4% of plan spending. A Brown University analysis found no statistically significant changes in hospital revenue or margins and no evidence of service-line closures, staffing cuts or clinician pay cuts. [30] Montana's earlier plan-level approach (220 to 250% of Medicare) was estimated to save $47.8 million over three years, but administration passed to a Blue Cross plan in 2022, showing the political durability problem. [30] Washington has adopted a similar cap. If every state employee plan had capped at 200% in 2022, savings would have averaged about $150 million a state, $7.1 billion nationally. [30]

> **Illustrative scale-up (PRISM arithmetic, untested).** Private insurers paid $558.9 billion for hospital care in 2024. [1] If commercial hospital prices average 254% of Medicare [18] and a 200% cap applied to every claim, average prices would fall by at least 21%, or about $119 billion a year, before exemptions for small hospitals, negotiation responses and cost shifting. That is an order of magnitude, not a forecast. Some states have gone beyond public plans to broader commercial price controls, but we did not retrieve evaluations of those programs. [30]

Competition enforcement and transparency. The evidence that concentration raises prices is strong (Section 4.3), which supports merger scrutiny and limits on facility-fee billing. Transparency by itself is a weaker lever: the employer-led price-transparency study notes many hospitals have not complied with federal price-posting rules, and workers moved to a high-deductible plan did not shop for lower prices even when given tools. [23, 18] We did not retrieve quantitative retrospectives of merger remedies.

Evidence **moderate to strong on prices, thin on remedies**

7.2 Drug pricing

Negotiation. Negotiated prices for the first 10 Part D drugs took effect January 1, 2026, at discounts of 38 to 79% off list prices, with CMS estimating $6 billion in Medicare savings in 2026 and $1.5 billion in lower out-of-pocket costs. [32] The second cycle (15 drugs, including Ozempic and Wegovy) takes effect in 2027, with an estimated $12 billion in savings, or 44% net, had the prices applied in 2024. A third cycle of 15 Part B and D drugs was selected in January 2026 for 2028. [32] A $2,000 Part D out-of-pocket cap began in 2025. [32]

PBM reform. A February 2026 law delinks Part D PBM pay from drug prices and rebates, requires PBMs to pass through all rebates to employer plans and adds reporting; CBO scores $2.1 billion in deficit reduction over 10 years, and the Part D provisions take effect in 2028. [33] The administration has also pursued voluntary pricing agreements, a direct-to-consumer TrumpRx site and Innovation Center "most favored nation" models, but their savings are not yet known. [33]

7.3 Fix payment rules that reward volume or coding

Medicare Advantage. MedPAC recommends addressing coding intensity, replacing the quality bonus program and setting more equitable benchmarks. [27] CBO lists a coding-related option at $155 billion over ten years. [28] CMS proposed nearly flat 2027 payment rates. [27] The insurer-backed Better Medicare Alliance argues MedPAC's estimates ignore program value. [27]

Global budgets and all-payer rate setting. Maryland pays hospitals fixed annual budgets across all payers. In 2019 to 2022 the model cut hospital spending 6.1% and

admissions 16.2%, and reduced Medicare spending by $689 million over 2019 to 2021, though its primary care program cost CMS about $96 million a year without offsetting savings. [29] The federal model concluded in 2025. [29] The results are real but small in dollars, and the approach depends on a federal waiver and state rate-setting capacity.

Accountable care organizations. The Shared Savings Program reported $2.4 billion in net savings in 2024 across 476 ACOs, after paying $4.1 billion in shared savings. [31] Two cautions: benchmarks are spending targets, not counterfactuals, and ACOs under new benchmarking rules had net savings of $143 per person against $241 for the others. [31] In the ACO REACH model, an independent evaluation found standard ACOs reduced net spending by an estimated 0.1%, not statistically significant, while high-needs ACOs cut gross spending 8.9% but net spending rose 9.1% after incentive payments. [31]

7.4 Restructure financing and reduce overhead

Single-payer and unified financing. Estimates diverge sharply. CBO's 2020 analysis found national health spending in 2030 could change by anywhere from a $0.7 trillion decrease to a $0.3 trillion increase depending on payment rates, cost sharing and long-term-care coverage, while federal subsidies would rise by $1.5 to $3.0 trillion, with high uncertainty. [34] A Lancet analysis estimated 13% savings, more than $450 billion a year; its lead author had informally advised the writers of the Senate bill it analyzed. [34] Mercatus attributes lower CBO estimates than other analyses mainly to assumptions that supply could not meet new demand and to lower payer administrative costs. [34] The design choices, not just the concept, determine the result.

Cap the employer tax exclusion. CBO has listed reducing preferential tax treatment among its health options, yielding between $240 billion and $540 billion over a decade (an older estimate), because the open-ended exclusion encourages more comprehensive coverage. [35] The ACA's 40% excise tax on high-cost plans was designed to counteract that incentive. [35]

Administrative standardization. Given the size of the administrative gap (Section 4.7), simplifying claims, prior authorization and payer rules should matter, but we did

not retrieve rigorous evaluations of specific standardization programs. The potential is inferred, not measured.

7.5 Levers with limited or contested payoff

- **Higher cost sharing.** It lowers spending by 20 to 30% in the RAND experiment and 12 to 14% in the employer natural experiment, but through less care of all kinds, and with harm to the sickest and poorest. [22, 23] With 34% of covered workers already at $2,000 or more, the lever is largely used. [6]
- **Malpractice reform.** The whole liability system is 2.4% of spending, so even large changes yield small savings. [24]
- **Prevention and obesity treatment.** Obesity costs are large (Section 4.8), but net savings from treatment depend on drug prices, uptake and durability; negotiated prices for Ozempic and Wegovy begin in 2027. [25, 32] We did not retrieve net-cost evidence.
- **Workforce and supply.** Loosening residency caps and scope-of-practice limits may ease labor price pressure, but we found no causal estimates of the size. [37]

Lever	Targets	Best available estimate	Evidence	Main obstacle
Site-neutral payment	Payment rules, consolidation	$156.9B over 10 years (CBO), about $16B a year [28]	**Scored, not yet enacted at scale**	Hospital opposition; rural access
Medicare Advantage payment fixes	Payment rules	$76B gap in 2026; CBO option $155B over 10 years [27, 28]	**Agency estimate, disputed by industry**	Plan lobbying; benefit trade-offs

Lever	Targets	Best available estimate	Evidence	Main obstacle
Hospital price caps	Prices, market power	Oregon about 4% of plan spending; $7.1B nationally for state plans [30]	**Quasi-experimental, one state**	Political durability; rural exemptions
Competition enforcement	Market power	Not quantified	**Strong on prices, thin on remedies**	Slow; legal burden
Drug negotiation	Drug prices	$6B (2026); $12B on a 2024 basis for cycle 2 [32]	**CMS estimates; too early to observe**	Limited to selected Medicare drugs
PBM reform	Drug supply chain	$2.1B over 10 years (CBO) [33]	**Enacted; effective 2028**	Implementation
Global budgets, all-payer	Payment rules, prices	Maryland: hospital spending -6.1%; $689M Medicare over 3 years [29]	**Federal evaluation**	Needs waiver and state capacity
ACOs	Payment rules	$2.4B net (2024) [31]	**Benchmark-based; independent evaluation mixed**	Benchmarks; selection
Single-payer financing	Administration, prices, insurance	NHE in 2030: -$0.7T to +$0.3T (CBO) [34]	**Modeled; high uncertainty**	Financing, transition, politics

Lever	Targets	Best available estimate	Evidence	Main obstacle
Cap the tax exclusion	Insurance design	$240B to $540B revenue over 10 years (older CBO) [35]	**Scored**	Workers' tax burden
Higher cost sharing	Demand	20 to 30% lower spending (RAND) [22]	**Randomized**, but harms the sick	Access; already widespread

Lever summary. Estimates are annual unless stated and come from different sources and definitions. "Estimated potential" is not realized savings. PRISM's evidence rating is its own synthesis.

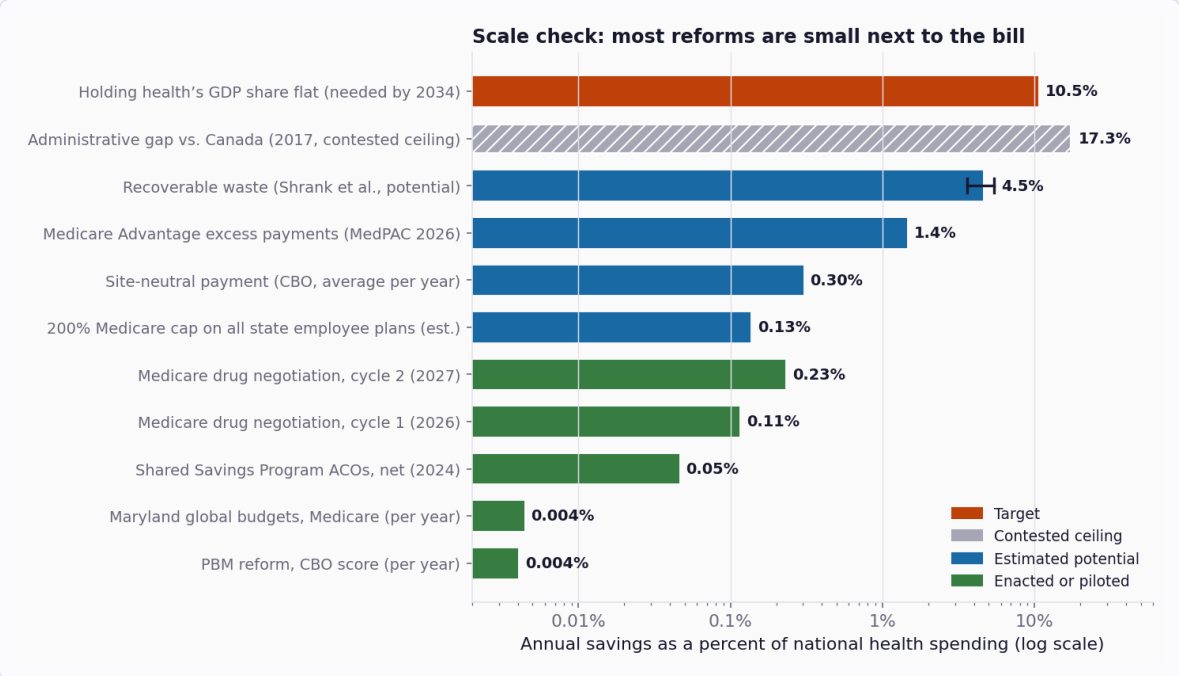


Figure 8. Scale check. Annual savings estimates as a percent of 2024 national health spending ($5,279 billion) on a log scale, against the reduction needed to hold health's GDP share flat by 2034 (10.5% of 2034 spending). Bases and definitions differ (some are Medicare-only), so bars are indicative, not additive. Sources: see Sections 3 and 6. [1, 4, 21, 27, 28, 29, 30, 31, 32, 33]

7.6 A plausible sequencing, and what would change our view

This is PRISM's reading of the evidence, not a recommendation for any program.

- **Tier A: evidence-backed and near-term.** Site-neutral payment, Medicare Advantage coding and benchmark fixes, state-level price caps for public plans, competition enforcement, and full implementation of drug negotiation and PBM delinking. They target the price and payment-rule drivers and have quantitative support.
- **Tier B: proven in pilots, harder to scale.** Global budgets or all-payer rate setting in more states, and administrative standardization across payers.
- **Tier C: system redesign with high uncertainty.** Single-payer or unified financing, and capping the employer tax exclusion. Estimates range from large savings to higher spending depending on design.

What would change this assessment. Realized results from the 2026 to 2027 negotiated drug prices; several years of Oregon and Washington data; Maryland-style models outside Maryland; an evaluation of most-favored-nation pricing; and any evidence that commercial price caps trigger provider exits or cost shifting.

8. The size of the prize

CMS projects national health spending to rise from $5.66 trillion in 2025 (18.4% of GDP) to $8.97 trillion in 2034 (20.6%). [4] That is growth of about 5.2% a year against about 4.0% for GDP. Holding health's share of GDP flat would need spending to grow with GDP, about 1.3 points a year slower.

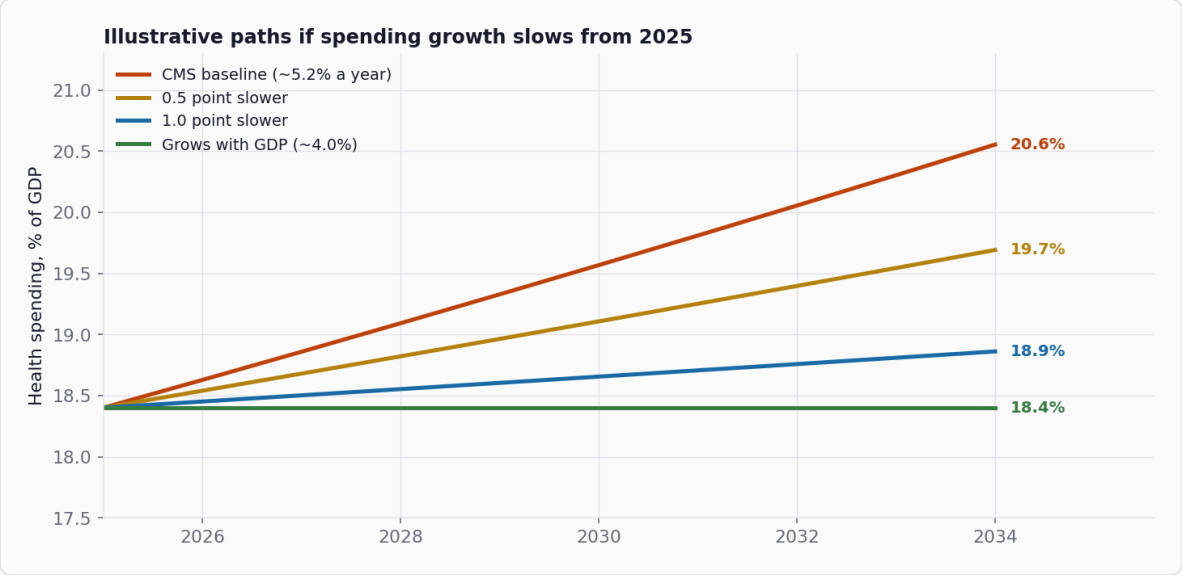


Figure 9. Illustrative paths for health spending as a share of GDP if annual spending growth slows from 2025. Constant-growth approximations anchored to CMS 2025 and 2034 values. [4]

Scenario	Growth a year	2034 spending	Share of GDP, 2034	Lower in 2034	Cum
CMS baseline	5.2%	$8.97T	20.6%	n/a	
0.5 point slower	4.7%	$8.59T	19.7%	$0.38T	
1.0 point slower	4.2%	$8.23T	18.9%	$0.74T	
Grows with GDP	4.0%	$8.03T	18.4%	$0.94T	

Illustrative constant-growth paths built from the CMS endpoints (2025 and 2034). PRISM arithmetic, not score. Baseline reproduces CMS's 2034 figure.

What the arithmetic implies. Bending growth by 1.3 points for nine years requires the level of spending to end about 10.5% below baseline. Single levers from Section 7 range from under 0.01% to 1.4% of spending. Even the estimated recoverable-waste range (3.6 to 5.4%) covers less than half of the gap, and the estimates overlap and are not additive. Closing the gap would need several large levers together, sustained for a decade, or a slower pace of technology-driven intensity growth. Two caveats: CMS projections have overshot before (Section 2), and the near-term slowdown CMS

expects in 2027 to 2028 partly reflects Medicaid enrollment loss rather than lower costs. [11, 12]

9. Red-team review and limits

Challenge	Finding	Status
Is the growth accounting robust?	It is deterministic arithmetic on CMS and BEA data, reproducible from the cited tables, but has no uncertainty intervals. The CMS price index holds the basket fixed, so quality and intensity change lands in "real use". The BLS hospital services CPI rose 15.7-fold from 1980 to 2024 against 5.4-fold for the CMS hospital price index, and the two are unreconciled, so the price share of growth may be understated.	Flagged in text
Can driver shares from different studies be added?	No. Windows (1960 onward, 1996 to 2013, 1980 to 2024) and definitions differ, and technology estimates are residuals.	Stated in method and scorecard
Are savings estimates realized or modeled?	Most are budget scores or agency estimates. ACO benchmarks are not counterfactuals, and an independent ACO REACH evaluation found incentives offset gross savings.	Flagged
Source bias	Waste estimate: authors reported Humana and UPMC ties. Administrative estimate: released through a single-payer advocacy group. Medicare Advantage: insurer-backed group disputes MedPAC. Single-payer: CBO, Lancet and Mercatus disagree. Each is presented with the counter-position.	Mitigated
Access to full texts	Several journal articles were read at abstract or reputable-summary level only, so subgroup findings and caveats may be missed.	Open limitation

Challenge	Finding	Status
Fast-moving policy	The ACA credit expiry, 2025 budget-law Medicaid changes, IRA cycles, the PBM law and most-favored-nation pricing are all recent. Figures reflect information available to 20 September 2026.	Re-check before reuse
Causality	Most evidence is observational. Randomized results (RAND) predate today's market and excluded people over 64. Consolidation studies control for confounders but cannot rule out all of them.	Hedged language used
Citation integrity	12 references with a DOI or indexed title resolved in Crossref. Agency and organization pages were retrieved by search; some facts come from secondary summaries and are labeled in the source list.	Verified where possible

Fact-Checker, Statistical Reviewer, Bias Auditor and Methodological Reviewer findings.

10. Conclusions

- Health spending has outgrown GDP for 40 years through a combination of more intensive care per person, higher prices than peers pay, insurance that mutes price signals, and heavy administrative overhead. No single factor dominates, and aging has been a minor historical contributor.
- The clearest lever candidates target prices and market power, payment rules that reward volume or coding, and administrative fragmentation. Demand-side cost sharing has mostly run its course and shifts costs to sicker people.
- Most enacted or piloted reforms are individually small. Bending the curve to GDP growth needs roughly a 10% lower level of spending by 2034, which implies stacking several large levers.
- Coverage losses can lower measured spending without lowering costs, so success should be judged on prices and volumes per covered person, not aggregate dollars.

Overall confidence: moderate High on levels and the price gap; moderate on driver shares; low on projected savings.

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